

Getting to the hospital Arrange for someone to drive you to the hospital, either your partner, or a friend or relative; don't consider driving yourself. Map the route ahead of time, consider a dry run before the big day, and have a bag packed with everything you will need for you and your baby (see [Items for your hospital bag](#)).

Admission procedure When you get to the hospital, you'll be checked in and will be put into a labor room if you look like you're in active labor, or into an assessment bed if this isn't clear. Usually you will be asked for a urine specimen, and a nurse will check your temperature, pulse, and blood pressure, check your cervix, and review your pregnancy history. If you're in early labor, you may be sent home. This doesn't mean you were unwise to come in; it's good to ensure all is well.

Once you've been admitted, you and your baby will be assessed by a doctor or nurse. Sometimes an intravenous (IV) line will be placed and blood tests may be done. You can make your room comfortable with items from home.

AS LABOR BEGINS

If you're having a home birth, your midwife will have talked to you in advance about how to contact her once you're in labor. This may be via her cell phone or via the birth center. Since she will be traveling to you, bear in mind local traffic conditions. If the streets might be busy, it's worth phoning her in early labor. She may ask you to phone again when your contractions are closer together.

While waiting for the midwife, you may want to move around, or relax in a warm bath. If you've purchased or rented a birthing pool, ensure that this is ready to use. Ask your partner to lay down old sheets or plastic sheeting over the floor. Eating small, nourishing snacks and drinking water will provide energy for the hours ahead.

Home births are not for everyone. They're only an option for you if you're having a healthy, low-risk pregnancy, and you may need to be transferred to a hospital before delivery if complications arise. The American College of Obstetricians and Gynecologists opposes home births because of the potential for complications, even in low-risk pregnancies, as does the American Medical Association. Only 6 out of every 1,000 U.S. births are home births.

Abdominal and vaginal examinations

You will have several internal vaginal examinations and your abdomen palpated to assess the baby's position. If your doctor is assessing whether amniotic fluid is leaking, they may do a speculum examination. Usually, the doctor uses their fingers to assess the baby and the progress of labor. They will try to check you often enough to make sure that your labor is progressing, but not so often that it causes you extra discomfort or increases your risk of infection. The following are assessed during a vaginal examination.

The station The doctor will check to see how far the head has descended into the pelvis (see [The "station"](#)).

Cervical effacement The doctor will assess that the cervix is shortening and getting ready for dilation (see [Changes to the cervix](#)).

Cervical dilation The doctor will assess how dilated, or open, your cervix is (see [Dilation](#)). Active labor is established at 3 to 4 cm dilation and full dilation occurs at around 10 cm. You can't push your baby out until you're fully dilated.

Fetal position Fetal presentation refers to the part of your baby that is coming out first. Babies can be born head first or bottom first ([breech](#)). Your doctor will also assess which way your baby is facing in the birth canal.